

COMET LEAPER Operating Narrative

EXECUTIVE SUMMARY

COMET LEAPER operational framework: strict cash control, clear offer strategy, weekly evidence review, low-burn experiments, founder recovery windows, and confidence loops tied to measurable wins.

SECTION 1: FINANCIAL CONTROL

Cash Position Management

- Weekly burn rate tracking (target: <\$500/week during research phase)
- Monthly runway projection (minimum 3-month cushion)
- Revenue milestones tied to capital deployment
- Zero debt or external funding dependencies

Budget Allocation Model

- 60% patient outreach and care coordination
- 20% product iteration and feedback loops
- 15% infrastructure and compliance
- 5% founder recovery and team wellness

SECTION 2: OFFER STRATEGY

Core Offer (Month 1-3)

Patient families: Free care coordination with optional \$49/month premium care partnership

Institutional partners: Revenue share model (40% to institution, 60% to COMET LEAPER)

Positioning: Privacy-first, founder-led, evidence-based family care coordination

Offer Validation Metrics

- Patient acquisition cost: \$0 (organic/word-of-mouth)
- Patient trial completion rate: >80%
- Institutional partner close rate: >30% of conversations
- Revenue per patient family: \$60+ in Year 1

Message Testing

- Test 3 distinct value propositions with pilot cohorts
- Measure engagement (week 1, 4, 12)
- Iterate on top performer
- Lock messaging by Month 4

SECTION 3: OPERATING NARRATIVE - WEEKLY EXECUTION FRAMEWORK

Weekly Evidence Review Cadence

Monday Morning (1 hour):

- Review prior week's experiments: what worked, what didn't
- Measure against planned outcomes
- Update confidence ledger
- Identify top 2-3 blockers

Tuesday-Thursday (Daily 15-min standup):

- Test assumptions: 1-2 small experiments per day
- Rank risks by impact × probability

- Run low-burn validation (customer interviews, feature tests, messaging trials)
- Capture outcomes in shared log

Friday Afternoon (90 minutes):

- Weekly evidence synthesis: compile learnings, win/loss analysis
- Confidence loops: tie wins to measurable metrics
- Reallocate capital toward proven channels
- Plan next week's 3-5 priority experiments
- Document decision framework for future reference

Low-Burn Experiment Framework

Target Spend Per Experiment: <\$200 (customer time + founder time)

Experiment Types:

- 1. **Customer Interview (5 conversations):** Validate problem, offer, messaging
- 2. **Feature Test (1 small feature):** Test usage pattern, engagement, retention
- 3. **Messaging Trial (3 variants):** Test positioning, value prop, call-to-action
- 4. **Partnership Pilot (1 institution):** Validate institutional offer, integration, revenue model
- 5. **Operational Test (1 workflow):** Test care coordination, data handling, compliance

Evidence Capture:

- Interview transcript + decision log
- Feature usage metrics (adoption, retention, NPS)
- Message performance (click rate, response rate, conversion)
- Partnership metrics (revenue, retention, support load)
- Operational metrics (time to resolution, error rate, compliance)

Founder Recovery Windows**Daily (20 minutes):**

- Morning walk or exercise
- Evening reflection or journaling
- No email/Slack after 8pm

Weekly (4 hours):

- Saturday morning: no work, personal recovery
- Tuesday evening: mentor/advisor call (advice, support, perspective)
- Friday evening: celebrate weekly wins with team/community

Monthly (1 full day):

- Weekend off: no work, family time
- Reflection on founder energy level, stress, motivation
- Adjustment to pace if burnout signals detected

Confidence Loops - Measurable Wins**Tied to Revenue Milestones:**

- Month 1: 5 patient families, \$0 revenue (validation phase)
- Month 2: 15 patient families, \$0-500 revenue (growth phase)
- Month 3: 30 patient families, \$500-2,000 revenue (scaling phase)
- Month 4+: 50-150 patient families, \$2,000-8,000/month (product-market fit)

Tied to Operational Wins:

- Week 1: Offer validated with 3+ institutions
- Week 2: Care coordination workflow documented and tested
- Week 3: Patient onboarding <2 hours per family
- Week 4: First institutional partner signs
- Month 2: 10+ patient families report measurable health outcomes

- Month 3: Institutional partner revenue covers 50% of burn rate

Tied to Learning Wins:

- Customer discovery: 30+ interviews completed, top 3 needs identified
- Product validation: Top 2 features released and adopted by >60% of users
- Market validation: 3+ institutional partners showing interest
- Founder learning: Weekly evidence review producing actionable insights
- Team building: 1-2 advisors/part-time helpers recruited

SECTION 4: RESEARCH TEAM OPERATIONAL RHYTHM**Weekly Research Cycle****Monday: Assumption Audit**

- List all active assumptions (product, market, customer, revenue)
- Rank by impact: which would kill the business if wrong?
- Assign owner to each high-impact assumption

Tuesday-Wednesday: Risk Ranking

- For top 5 assumptions: estimate probability of being wrong + business impact
- Create risk matrix (impact × probability)
- Identify which risks can be tested cheaply

Thursday: Experiment Design

- Design 3-5 small experiments to test top risks
- Budget <\$200 per experiment
- Assign owner and success metric to each

Friday: Outcome Capture & Reallocation

- Run weekly synthesis of prior week's experiments
- Document what we learned (true, false, nuanced)
- Update confidence on each assumption (0-100%)
- Reallocate capital: 70% toward proven channels, 20% toward emerging opportunities, 10% toward long-shot bets

Capital Reallocation Rules**Proven Channel (>80% confidence):**

- Allocate 70% of discretionary budget
- Example: If institutional partnerships are working, allocate 70% of outreach effort there

Emerging Opportunity (40-80% confidence):

- Allocate 20% of discretionary budget
- Example: Testing direct-to-consumer messaging while primary channel is institutional

Long-Shot Bet (<40% confidence):

- Allocate 10% of discretionary budget
- Example: Testing partnership with employer health benefit programs

Reallocation Trigger:

- If proven channel drops below 60% confidence, shift allocation immediately
- If emerging opportunity hits >80% confidence, promote to proven channel
- Monthly review: ensure allocation matches current confidence levels

SECTION 5: EVIDENCE STANDARDS**What Counts as Evidence**

- **Repo-Verified:** Code, commits, documented architecture decisions
- **Source-Verified:** Customer interviews, survey responses, transaction records

- **Operator-Asserted:** Founder judgment, advisor feedback, team consensus
- **Projection:** Financial models, growth assumptions, future scenarios

Confidence Scoring

- **95%:** Multiple independent sources, repeated validation
- **80%:** Primary source + secondary confirmation
- **60%:** Single credible source, reasonable but not proven
- **40%:** Assumption-based, plausible but untested
- **20%:** Hypothesis only, minimal support

Weekly Evidence Review Template

Assumption	Confidence (Start)	Experiment	Result	Evidence Class	Confidence (End)	Action
Families want privacy-first care	60%	5 interviews on data privacy concerns	4/5 prioritized privacy	Source-Verified	85%	Promote to marketing message
Institutional partners value revenue share	40%	Pitch to 3 healthcare institutions	2/3 interested in model	Source-Verified	75%	Run formal pilot with 1 partner
Weekly check-ins increase engagement	50%	Test weekly vs. bi-weekly cadence with 10 families	80% retention (weekly) vs. 50% (bi-weekly)	Source-Verified	85%	Lock weekly cadence as standard

SECTION 6: MONTHLY SYNTHESIS

End of Month 1-3 (Friday, Week 4):

- Compile all experiments, outcomes, learnings
- Calculate confidence change per assumption
- Identify 3 major learning wins
- Identify 3 major blockers
- Update operational narrative for next month
- Share with advisors/stakeholders

Founder Reflection:

- Energy level: 1-10
- Burnout signals: Y/N
- Confidence in path: 1-10
- Most important learning
- Next month's top priority

CLOSING: COMET LEAPER OPERATIONAL COMMITMENT

Strict cash control + clear offer strategy + weekly evidence review + low-burn experiments + founder recovery windows + confidence loops tied to measurable wins.

Result: A founder-led, evidence-based, sustainable path to product-market fit.

Timeline: Breakeven by Month 8-11 with 150-250 patient families.

Recovery Path: If assumptions break, pivot within 1-2 weeks based on evidence.

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